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New Jersey's Direct Support Professionals

Concerned. Caring. Committed.

- As Direct Support Professionals
  SAFETY is your first concern.

- As Caregivers for adults with developmental disabilities
  PROTECTION from harm is your first priority.

You work together to prevent abuse, neglect and exploitation.

You are a lifeline for adults with developmental disabilities. You work to keep individuals safe from harm and you report any potential risk of abuse, neglect or exploitation immediately. Follow your facility, agency or program's reporting policies. Others can call 1-800-832-9173.



New Jersey's Stephen Komninos' Law reinforces the commitment to keeping adults with developmental disabilities healthy, safe and protected from harm. The Department of Human Services, the Division of Developmental Disabilities, and community providers of services for adults with developmental disabilities work together to improve safety in New Jersey's facilities, programs and services for adults with developmental disabilities.

Produced by the NJ DHS

IT'S THE LAW

Professionals and volunteers who abuse, neglect or exploit individuals with developmental disabilities may be added to The Department of Human Services, Central Registry of Offenders.






Caregivers serving individuals with a developmental disability could lose their jobs and be banned from future employment.

REPORT ABUSE

by calling 1-800-832-9173

State of New Jersey
Phil Murphy, *Governor*
Sheila Oliver, *Lt. Governor*

Department of Human Services
Carole Johnson, *Commissioner*

DDD PREVENTION BULLETIN | PRESSURE ULCERS



Pressure Ulcers (decubitus ulcers) are also called bed or pressure sores, which are damage to the skin caused by pressure. They often look like a minor redness in the skin although they can initially hide more damage under the skin's surface. Pressure ulcers can be painful and they can become infected sometimes causing blood poisoning or bone infections.



- In severe cases, they can spread deeply under the skin and some of the underlying muscle or bone may be destroyed
- Individuals most at risk of developing pressure ulcers include those who are confined to a bed or chair and unable to move independently, have poor circulation, have frequent moist skin, are elderly or with poor food, and fluid intake, and have a prior history
- The signs to look for especially in the areas where bones are close to the skin are red/purple/blue skin, blister, swelling, dryness or patches, shiny areas, cracks, calluses, and wrinkles
- The signs to feel for are hard or warm areas
- Untreated pressure sores can become infected and potentially result in sepsis

Pressure Ulcer Prevention

- Encourage individuals to move at least every hour, and if they depend on you, change their position at least every 15 minutes in a chair and every two hours in bed. Avoid friction (from rubbing, dragging) and shear (sliding down the bed)
- Provide adequate hydration and a balanced diet
- Assess the skin daily. Pay special attention to the feet. Those with decreased circulation may not feel pain
- Keep skin clean and dry at all times. Prevent moisture from sitting on the skin over a period of time as it allows bacteria to grow. Dry skin gently to prevent damage. Avoid massaging skin over bony parts of the body
- Bathe or shower in warm (not hot) water using a mild cleanser or soap
- Use a moisturizing lotion to prevent skin from drying out and cracking
- Make use of pillows and other products like cushions, pads, and mattresses to relieve pressure
- ***Ensure physician orders for repositioning guidelines are followed.*** Use the guidelines to create a repositioning schedule. Make sure all caregivers are trained on the schedule. It is advisable to maintain a repositioning logbook for staff awareness and pressure ulcer healing progress. This will help determine if the repositioning guidelines need to be adjusted by the physician

DDD PREVENTION BULLETIN | **PRESSURE ULCERS**

The Four Stages of Pressure Ulcers

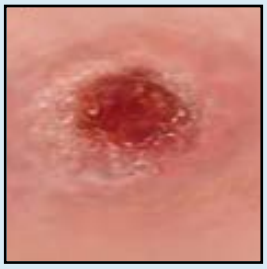
Pressure ulcers are grouped into four stages from stage one to four according to depth and severity, with stage four being the most severe. Bedsores in early stages 1& 2 may heal with relief of pressure and good skin care.

Stage One



- The skin is not broken
- Skin appears red on people with lighter skin color and does not briefly lighten (blanch) when touched
- On people with darker skin, the skin may show discoloration, and it does not blanch when touched
- The site may be tender, painful, firm, soft, warm, or cool compared with the surrounding skin

Stage Two



- The outer layer of skin (epidermis) and part of the underlying layer of skin (dermis) is damaged or lost
- The wound may be shallow and pinkish or red. It may look like a fluid-filled or a ruptured blister

Stage Three



- At this stage, the ulcer is a deep wound. The loss of skin usually exposes some fat
- The bottom of the wound may have some yellowish dead tissue
- The damage may extend beyond the primary wound below layers of healthy skin

Stage Four



- A stage IV ulcer shows large-scale loss of tissue. It may expose muscle, bone, or tendons
- The damage often extends beyond the primary wound below layers of healthy skin
- The bottom of the wound likely contains dead tissue that is yellowish or dark and crusty

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

DDD PREVENTION BULLETIN | **SEPSIS/INFECTION**

Signs and Symptoms:

- Signs of an infection
- High heart rate, low blood pressure
- Fever, feeling cold or shivering
- Confusion or disorientation
- Shortness of breath
- Extreme pain or discomfort
- Clammy or sweaty skin



Complications:

- **THE MOST SEVERE COMPLICATION** is Septic Shock! The body goes into shock from the decrease of blood pressure, in turn, causing decreased blood flow to tissues/organs possibly resulting in death.



Treatment:

- I V fluids
- Antibiotics, if the cause is bacterial
- Oxygen
- Additional treatments may be needed depending on the severity

Prevention:

- Prevent infections by taking care of chronic conditions and getting recommended vaccines
- Treating infections quickly, do not delay treatment!
- Have good hygiene by washing hands, keeping any cuts clean and covered, and overall good personal hygiene
- Routine catheter changes
- Evaluation of possible aspiration by swallow study
- Follow prescribed wound care regimen and repositioning guidelines



Facts about Sepsis:

- At least 1.7 million adults in America develop sepsis annually.
- Nearly 270,000 Americans die because of sepsis every year.
- 1 in 3 people who dies in a hospital had sepsis during that hospitalization.
- Sepsis, or the infection causing sepsis, starts outside of the hospital in nearly 87% of cases.

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

State of New Jersey
Department of Human Services
Division of Developmental Disabilities

Division of Developmental Disabilities

DDD PREVENTION BULLETIN

Choking



Choking is a major cause of medical injuries, but it can be prevented. This bulletin alert contains critically important information for caregivers of individuals with developmental disabilities. It provides information on signs of choking, food commonly connected to choking incidents, risk factors (including diagnosis and eating style), actions to take and preventative measures.

Choking is a blockage of the upper airway by food or an object that prevents someone from breathing. It is a medical emergency that requires fast action, to include an immediate call to 911, followed by efforts to dislodge the object that is causing the choking.

Why are individuals with developmental disabilities at risk of choking?

People with developmental disabilities are more likely to have a number of common characteristics that place them at high risk for choking/aspirating. Some of these characteristics include:

- Decreased or absent protective airway reflexes
- Poor oral motor skills that do not permit adequate chewing or swallowing
- Medication side effects
- Impaired mobility/positioning

Other risk factors include medical conditions such as:

- Dysphagia
- Asthma
- Lung disease
- Emphysema
- Sleep apnea
- Seizures
- Gastroesophogeal reflux disorder (GERD)

Signs of choking:

- Inability to talk
- Wide-eyed panicked look on face
- Clutching the throat or chest
- Inability to cough forcefully
- Difficulty breathing or noisy breathing
- Loss of consciousness
- Skin, lips, or nails turning blue or dusky

Common causes of choking:

- Eating or drinking too fast
- Not chewing food well enough prior to swallowing
- Distracted while eating (laughing/talking/moving with food in mouth)
- Distracted by other persons or activities
- Over stuffing mouth
- Food stealing
- Swallowing inedible objects (Pica)
- Incorrect diet texture - liquids or food items
- Poorly fit dentures
- Edentulous (having no teeth)
- Inadequate supervision
- Inadequately trained staff
- Not familiar with prescribed diet
- Poorly assisted eating techniques
- Allowing poor positioning

Special Risks for People with Swallowing Disorders

Food that is difficult to chew or swallow because of its shape, size, or texture further increases the risk of choking. People with swallowing disorders should avoid these foods:

- **Hot Dogs**
- **Tossed Salads**
- **Chicken on bone**
- **Marshmallows**
- **Meat Chunks**
- **Canned Fruit**
- **Hard Candy**
- **Raw vegetables (carrot, celery, etc.)**
- **Potato/Corn/ Tortilla Chips**
- **Nuts And Nut butters (e.g., peanut butter)**
- **Popcorn**
- **Raisins**
- **Grapes**
- **Bagels or other chewy bread**
- **Raw Hard Fruits (apple, grapes, etc.)**
- **Rice**
- **Pretzels**
- **Hard Beans**
- **Chewing Gum**
- **Caramel**

What to do if someone chokes:

Anyone unable to cough forcefully, speak or breathe may be choking:

- Immediately call 911
- Follow agency policy on Life Threatening Emergencies (Danielle's Law)
- If trained, respond to an individual who is choking by following the guidelines of the organization by which you were trained:
  - American Red Cross – five back blows, five abdominal thrusts and continue until medical personnel arrive and take over, the object is dislodged or the individual becomes unconscious
  - American Heart Association – perform abdominal thrusts continue until medical personnel arrive and take over, the object is dislodged or the individual becomes unconscious
- If the individual becomes unconscious, guide them to the floor and begin CPR if trained
- Notify primary care physician after the emergency is under control

Preventative measures:

- It is advisable to maintain a phone in the room during mealtimes
- Post information on how to communicate to 911 operators near phone
- Educate caregivers on the importance of mealtime safety
- Improve accountability by assigning point people for mealtime monitoring
- Closely monitor people who eat too fast and overstuff their mouth. Document mealtime concerns in the communication log
- Follow prescribed diets
- Promptly communicate diet changes to all settings
- Provide administrative oversight during mealtimes
- Concerns should be reviewed by appropriate personnel to determine if an assessment of eating/swallowing skills is needed, if mealtime monitoring strategies are needed, if prescribed diet needs to be modified or if adaptive feeding equipment is needed
- Be cautious of leaving food items accessible that could pose a choking risk

DISCLAIMER

This material is being provided strictly to alert staff of DDD and DDD's providers, families, individuals, and the general public about the risks of choking. At no time is this alert to substitute for training nor is it to suffice as training. Staff who have been trained in how to properly administer to an individual who is choking are expected and required to use their training to assist those in need.

Olmstead Resource Team

CMS

Physical Nutritional Management

O.O.I.E. NJ Office of the Ombudsman for the Institutionalized Elderly

Produced by DHS Office of Publications 7/2014

DDD PREVENTION BULLETIN
GASTROESOPHAGEAL REFLUX DISEASE (GERD)



Gastroesophageal Reflux Disease, or GERD, is a very common disorder. Gastroesophageal refers to the stomach and the esophagus. Reflux refers to the back-flow of acidic or non-acidic stomach contents into the esophagus. It occurs when the esophageal defenses are overwhelmed by stomach contents that reflux into the esophagus. A band of muscles at the junction of the stomach and esophagus called the lower esophageal sphincter (LES) normally acts, in conjunction with the diaphragm, as a barrier to prevent reflux of stomach contents into the esophagus. If that barrier is relaxed at inappropriate times or is otherwise compromised, reflux occurs.

There is no known single cause of GERD, however, lifestyle or behavioral factors can stress the LES and contribute to its failure. These factors include:

- Obesity
- Smoking
- Alcohol use
- A high-fat diet
- Consumption of carbonated beverages



People with IDD with the following diagnosis are at higher risk:

- GERD is very common, affecting up to 1 in 5 or more adult men and women in the U.S. population. It also occurs in children. Although common, the disease often is unrecognized – its symptoms are misunderstood. This is unfortunate because GERD is generally a treatable disease, though serious complications can result if not treated properly.

People with developmental disabilities are at high risk for GERD due to some of these characteristics:

- Dysphagia Diagnosis
- Anatomic abnormalities
- Impaired Mobility/ Immobility
- Need for Positioning/ Repositioning
- Dependency on others for feeding
- Medication side effects



DDD PREVENTION BULLETIN | **GERD**

Signs and Symptoms:

- Heartburn is the most frequent, which is a burning discomfort that begins behind the breastbone and radiates to the neck and throat.
- Acid regurgitation (refluxed material into the mouth)
- Bad breath
- A sour taste
- Belching
- Difficulty or pain when swallowing
- Water brash (sudden excess of saliva)
- Dysphagia
- The sensation of food sticking in the esophagus
- Chronic sore throat
- Laryngitis
- Inflammation of the gums
- Erosion of the enamel of the teeth
- Chronic irritation in the throat
- Hoarseness in the morning





Treatment:

It is essential for individuals who suffer the chronic and recurrent symptoms of GERD to seek an accurate diagnosis, work with their physician, and receive the most effective treatment available. Various methods to treat GERD range from lifestyle measures to the use of medication or surgical procedures.

Prevention and Management of GERD:

Reduction of symptoms can be achieved in most individuals with several behavioral and lifestyle modifications. These include:

- Weight loss
- Avoidance of carbonated beverages
- Smoking Cessation
- Reducing alcohol and caffeine intake
- Avoiding “trigger” foods (spicy foods, citrus, or acidic foods)
- Maintaining a low-fat diet
- Avoiding eating or drinking several hours before going to bed
- Remaining upright immediately after eating
- Elevating the head of the bed at night

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

PICA

Pica is defined as the compulsive and recurrent consumption of non-nutritive/non-food items. The term pica is derived from the Latin word for the bird "magpie", a bird known for consuming food and non-food items alike. A pica event occurs when the non-food item is swallowed/ingested.

Pica Attempt

A pica attempt occurs when an individual tries to ingest (swallow) the non-nutritive/non-food Nstaff before ingestion occurs. Both pica and pica attempts involve intent to swallow the non-food item.

Mouthing

Mouthing items is not considered pica. It involves placing non-food items in the mouth and manipulating them without the intent to ingest the item.

When is Pica suspected?

- ❑ Non-food items are consumed repeatedly despite efforts to curtail the behavior.
- ❑ The behavior is considered inappropriate for the individual's developmental and/or chronological age.
- ❑ It is not an accepted cultural practice.
- ❑ The behavior is a symptom of another mental health disorder and is of sufficient concern to warrant medical attention.



Pica can occur due to medical conditions such as anemias, gastrointestinal concerns such as GERD, use of G-tube, or dialysis.

Pica can occur during the course of another mental health disorder (e.g., Schizophrenia, Borderline Personality Disorder, etc), and can warrant independent clinical attention.

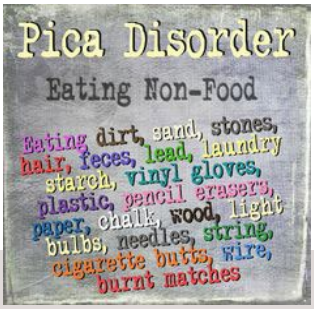
Pica can be a learned behavior that achieves a specific result for the person when he/she engages in the practice.

State of New Jersey
Department of Human Services
Division of Developmental Disabilities

Prepared by DHS Office of Publications 11/2016

Pica and Developmental Disabilities/ Intellectual Disabilities

Pica is observed with a high frequency in individuals diagnosed with DD/ID.





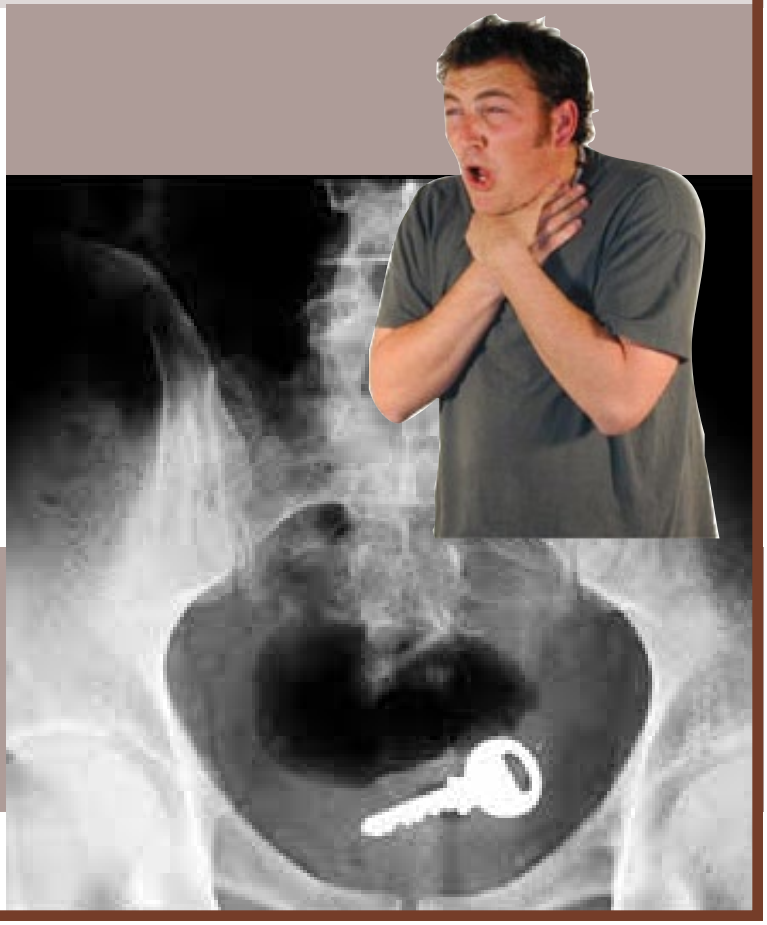
Possible reasons for occurrence of Pica among the DD/ID population

- ❑ Medical Issues
- ❑ Learned behavior
  - ❑ Inability to discriminate food and non-food items
  - ❑ Other psychiatric conditions
  - ❑ Mineral deficiencies
  - ❑ Dietary/Caloric intake deficiencies
  - ❑ Loss of opportunity to eat by mouth
  - ❑ Dementia

Risks From Pica

- ❑ Dental/Mouth Injuries
- ❑ Choking/Aspiration
- ❑ Ulceration and Perforation of esophagus and/or stomach
- ❑ Poisoning
- ❑ Bowel Blockage





Management and Precautions

- ❑ Assess Pica with a medical team first and foremost to rule out medical issues.
  - o If it is a medical issue, ensure appropriate treatment begins as soon as possible.
- ❑ Assess Pica from with a behavioral team after medical rule out has been completed and cleared.
  - o If Pica is a behavioral concern, it will require a functional behavior assessment to pinpoint the function/reason of the behavior. This will help to ensure that appropriate treatment is identified as well we as appropriate functional replacement behavior. Additionally, functional communication training may be put in place and taught to the individual.
- ❑ Environmental strategies can be implemented to prevent occurrence of pica.
  - o Discuss any and all changes to environment with the appropriate Human Rights Committee (HRC) and/or Behavior Management Committee (BMC) for approvals.
  - o Identify item(s) that are ingested
  - o Alert all who work or live with individual of pica diagnosis and inform them of items that the person ingests or is likely to ingest.
  - o Clear the environment of the items that the person is known to ingest (with HRC approval).
  - o Block and redirect pica attempts using approved and trained strategies by agency.
  - o Individual may require additional strategies with approval of Interdisciplinary Team and Agency.
  - o Individual may require a behavior support plan.



What to do if Pica event occurs

- ❑ **If individual ingests non-food item(s), medical assistance must be accessed, call 911.**
- ❑ **An unusual incident report must be completed.**
- ❑ **Communicate details concerning the event to medical professionals and agency staff.**

Sources

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DDD PREVENTION BULLETIN | **ASPIRATION**

Tips for Decreasing the Risk of Aspiration



- *The individual should be seated at a 90 degree angle for all oral intake, or as recommended by a specialist.*
- *The individual should take small bites of indicated diet texture (1/2-3/4 teaspoon).*



- *The individual should take small sips of indicated beverage consistency.*
- *If thickened liquids are prescribed, ensure that ALL liquids are thickened (drinks, soups, cereals).*



- *The individual should eat at a slow rate of intake.*
- *The mouth should be completely empty before taking another bite or sip.*
- *This individual should avoid talking with food in their mouth.*



- *Staff should ensure an unrushed atmosphere that allows for independence.*

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

DDD PREVENTION BULLETIN | SEIZURES



Seizures are episodes of disturbed brain activity which can affect one part or multiple parts of the brain. A seizure happens when nerve cells in the brain don't function properly and there is a sudden abnormal electrical signal in the brain. Seizures vary in appearance and severity depending on where they start in the brain.

Epilepsy is a disorder of the brain, characterized by recurrent seizures (uncontrolled body movements). Epilepsy is a common childhood neurological condition that is associated with many developmental disorders.

Why people with a developmental disability may be at higher risk for seizures:

- *Individuals with developmental disabilities are more likely to have seizures or epilepsy due to underlying brain conditions.*



Risk Factors for Seizures:

- Progressive brain disease
- Head trauma
- Hypoglycemia
- Congenital conditions
- Brain Tumor(s)
- Genetic factors
- Stroke
- A history of seizures within the last 5 years

Signs and Symptoms of a Seizure(s):

Symptoms vary from person to person, depending on the type of seizure. Some have staring spells while others may shake violently. Seizures are categorized as either generalized (involving the entire cerebral cortex) or partial (involving part of one cerebral hemisphere). Seizure symptoms can include:

- Rapid eye blinking or staring
- Twitching of the face
- Odd repetitive behavior
- Shaking or jerking of the limbs
- Stiffening of the body
- Sudden aggressive behavior



DDD PREVENTION BULLETIN | **SEIZURES**

Treatment and Prevention:

- Seizure medication should be administered/taken as prescribed (correct dosage and time)
- The individual should have regular follow-up with a Neurologist
- Records should be kept of any seizure activity: how long seizure lasted and a description of each seizure





Management and Precautions:

- Staff should check to make sure the individual is breathing
- Staff should help the individual lie down on the bed or floor, loosen clothing around the neck and remove eyeglasses
- Staff should stay with an individual during a seizure:
  - Staff should not place anything in the person's mouth or restrain them
  - Staff should move objects away to prevent injury
  - Staff should, if possible, provide padding under individual's head/arms/legs
  - Staff should only move the individual if the area is unsafe
- After the seizure has ended, staff should ensure the person is comfortable, allow quiet time to recover from the seizure and check on the individual every 15 minutes to make sure he/she is breathing normally



Call 911 for emergency help if someone is having a seizure and:

- It is the first time the person has had a seizure
- The person has stopped breathing
- The seizure lasts 5 minutes or longer
- The person has another seizure soon after the first one stopped
- The person is not fully awake within a few minutes after the seizure
- The person's lips or face look blue
- The person falls and hits their head during a seizure
- A seizure happens after a head injury

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

Division of Developmental Disabilities
Resource Team: Nursing Support Unit
Seizure Observation Form

Staff Initials	Signature	Staff Initials	Signature	Staff Initials	Signature

Notes:

_____

_____

_____

_____

_____

_____

*Tips for Seizure Observation and Recording	
Behavior before the seizure: • What was the person doing? • Change in mood or behavior?	**Possible Triggers:** • Time of day • Menstruation or Pregnancy • Missed or a change in medication • Irregular sleep or eating patterns • Alcohol or other drug use • Stress/Excitement • Sounds/Flashing lights • Illnesses
What happens during: • Change in Alertness/Mental Status • Ability to talk and understand • Eye twitching/blinking/rolling • Body stiffness/limp • Muscle jerking/twitching • Automatic/ Repeated movements • Change in color of skin • Loss of urine or bowel control	**What happens after:** • Awareness of name, place, and time • Ability to talk or communicate • Weakness or numbness • Change in mood • Wakefulness **Documentation:** • Length of seizure • Length of recovery

Reference: Joseph I. Sirven MD, Patty Obsorne Shafer RN, MN, Steven C, Schachter, MD, Epilepsy Foundation, accessed 24 April 2024, https://www.epilepsy.com/manage/tracking/observation

If a life-threatening emergency is present always call 911 as per Danielle's Law.

DDD PREVENTION BULLETIN | **DEHYDRATION**



Dehydration is a loss of too much fluid from the body. The body needs water in order to maintain normal functioning. If your body loses too much fluid - more than you are getting from your food and liquids - your body loses electrolytes.

Electrolytes include important nutrients like sodium and potassium which your body needs to work normally. A person can be at risk for dehydration in any season, not just the summer months. It is also important to know that elderly individuals are at heightened risk for dehydration because their bodies have a lower water content than younger people.

Why people with a developmental disability may be at a higher risk for dehydration:

- People with physical limitations may not be able to get something to drink on their own and will need the assistance of others.
- People who cannot speak or whose speech is hard to understand may have a hard time telling their support staff that they are thirsty.
- Some people may have difficulty swallowing their food or drinks and may refuse to eat or drink. This can make them more susceptible to becoming dehydrated.



Common Causes and Risk Factors for Dehydration:

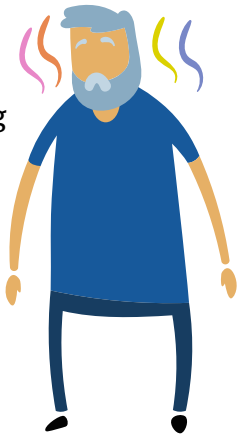
- Diarrhea
- Vomiting
- Excessive sweating
- Fever
- Burns
- Diabetes when blood sugar is too high
- Increased urination (undiagnosed diabetes)
- Not drinking enough water, especially on warm and hot days
- Not drinking enough during or after exercise
- Some medications (diuretics, blood pressure meds, certain psychotropic and anticonvulsant medications)



DDD PREVENTION BULLETIN | **DEHYDRATION**

Most Common Signs and Symptoms of Dehydration:

- Dry mouth and tongue with thick saliva
- Dizziness
- General weakness
- Flushed face
- Dark, concentrated urine
- Confusion in older adults
- Few or no tears when crying
- Flushed face
- Headache
- Sleepy, difficult to wake up
- Decreased urination
- Dry, warm skin
- Decreased skin elasticity



- One test for dehydration is to lightly pinch the skin on the back of the hand. If it is slow going back into place, this may indicate dehydration. Mild to moderate dehydration can be corrected by drinking more water alone, but if severe dehydration develops, it can be a medical emergency.

In Severe Dehydration these additional symptoms will be present:

- Sunken eyes
- Low blood pressure
- A lack of sweating
- Rapid heart beat



Best Ways to Keep Hydrated and Avoid Dehydration:

- Always carry a water bottle for each person in the group when going out in the sun.
- Offer drinks throughout the day (not only at meals). People should have 8 to 10 glasses of fluid every day unless otherwise indicated.
- Soda is not a good choice for rehydration because it contains too much sugar and not enough sodium to replace electrolytes.
- Increase intake of foods with high water content, such as fruits and vegetables.
- Encourage snacks like popsicles, Jell-O, diced fruit cups, apple sauce, and Italian ice since these have high water content.
- Increase fluid intake at the first signs of illnesses such as a cold or the flu.
- Use an air conditioner or fan in hot weather. Adults should never stay in temperatures over 100°F (37.8°C), especially indoors, without good air flow.

If you have any questions regarding this material or would like to schedule a training, please contact The DDD Resource Team at ddd.resourceteam@dhs.nj.gov.

DDD PREVENTION BULLETIN | SEPSIS/INFECTION



Sepsis is the body's extreme and life-threatening response to an infection. Sepsis happens when an infection you already have triggers a chain reaction throughout your body. Infections that lead to sepsis most often start in the lung, urinary tract, skin, or gastrointestinal tract. Without timely treatment, sepsis can rapidly lead to tissue damage, organ failure, and death.

Bacterial infections are the most common causes of sepsis. Fast recognition and treatment can increase the chances of survival

Sepsis/Infection Risk Factors:

- Organ transplant recipients
- Recent history of invasive surgical procedures
- Severe illness or hospitalizations
- Poor or dependency on others for hygiene
- Dependency with feedings
- Anatomic abnormalities
- Immobility causing pressure ulcers
- Indwelling urinary catheters, G-tubes, shunts, tracheostomies, and other indwelling devices





People with developmental disabilities are at high risk for Sepsis due to some of these diagnoses:

- Aspiration
- Dysphagia
- GERD
- Weakened immune systems due to autoimmune disorders or malnutrition
- Chronic medical conditions (diabetes, hepatitis, cancer, kidney disease)
- Constipation
- Prior sepsis history
- Epilepsy
- Cerebral Palsy
- Down syndrome

DDD PREVENTION BULLETIN | **SEPSIS/INFECTION**

Signs and Symptoms:

- Signs of an infection
- High heart rate, low blood pressure
- Fever, feeling cold or shivering
- Confusion or disorientation
- Shortness of breath
- Extreme pain or discomfort
- Clammy or sweaty skin



Complications:

- **THE MOST SEVERE COMPLICATION** is Septic Shock! The body goes into shock from the decrease of blood pressure, in turn, causing decreased blood flow to tissues/organs possibly resulting in death.



Treatment:

- I V fluids
- Antibiotics, if the cause is bacterial
- Oxygen
- Additional treatments may be needed depending on the severity

Prevention:

- Prevent infections by taking care of chronic conditions and getting recommended vaccines
- Treating infections quickly, do not delay treatment!
- Have good hygiene by washing hands, keeping any cuts clean and covered, and overall good personal hygiene
- Routine catheter changes
- Evaluation of possible aspiration by swallow study
- Follow prescribed wound care regimen and repositioning guidelines



Facts about Sepsis:

- At least 1.7 million adults in America develop sepsis annually.
- Nearly 270,000 Americans die because of sepsis every year.
- 1 in 3 people who dies in a hospital had sepsis during that hospitalization.
- Sepsis, or the infection causing sepsis, starts outside of the hospital in nearly 87% of cases.

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